

### Core Principles for TSS:

1. Recognize the syndrome: Fever plus hypotension plus rash plus multi-organ failure.
2. Immediate management: Aggressive fluid resuscitation plus broad-spectrum antibiotics.
3. Add clindamycin to inhibit toxin production (it inhibits protein synthesis and suppresses superantigen production).
4. Source control: Remove tampon/packing, debride wound.

## Community-Acquired Pneumonia (CAP)

- Fever, cough (often productive), dyspnea, pleuritic chest pain. Physical examination shows tachypnea and crackles.

### DIAGNOSTIC PATHWAY

- **Required for diagnosis:** Chest X-ray demonstrating a new infiltrate.
- Sputum and blood cultures plus urine antigen tests (*S. pneumoniae* and *Legionella*) are recommended for hospitalized or severe cases.

### Core Principles for CAP Management:

1. **Outpatient (healthy):** Amoxicillin or doxycycline.
2. **Outpatient (comorbidities):** Combination therapy (amoxicillin-clavulanate plus azithromycin or doxycycline) or monotherapy (levofloxacin).
3. **Inpatient (non-ICU):** Ceftriaxone plus azithromycin, or a respiratory fluoroquinolone if beta-lactam allergy.
4. **Inpatient (ICU):** Ceftriaxone plus azithromycin (or a fluoroquinolone). Add coverage for *Pseudomonas* (piperacillin-tazobactam or cefepime) and MRSA if risk factors are present.
5. Use severity scores (CURB-65 or PSI) to guide admission decisions.

**PEARL:** CAP disposition (CURB-65): Confusion, urea >19 mg/dL, RR ≥30, BP <90/60, age ≥65. 0–1: outpatient; 2: consider admit; ≥3: admit (often ICU).

## Aspiration Pneumonia and Aspiration Pneumonitis

### KEYWORDS / TYPICAL PRESENTATION

- Patient with dysphagia, altered mental status (stroke, seizure, alcohol intoxication), recent general anesthesia, or poor dentition who develops respiratory symptoms.
- **Aspiration Pneumonitis (Chemical):** Acute respiratory distress within hours of witnessed aspiration. Inflammatory response to gastric contents, NOT infection. Often improves in 24-48 hours.
- **Aspiration Pneumonia (Infectious):** Develops days after aspiration event (often unwitnessed). Infection from oropharyngeal flora.